Alcohol and Polysubstance Use in Chapel Hill - Qualitative Research Findings and Recommendations Report

Gabriel Bennett, Salma Treish, Alexis Valerius, and A Wheeler

UNC Gillings School of Global Public Health MPH Capstone

Campus and Community Coalition

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Introduction

Alcohol, drug use, and polysubstance use are widespread on college campuses and result in significant consequential behaviors. A total of 59.9% of surveyed undergrads in the United States in 2024 reported alcohol use in the past month. Alcohol misuse among college students is linked to lower academic performance, poor mental health, and safety risks (Martin et al., 2021). Drinking during young adulthood can also impact brain development and function, leading to issues such as impaired memory formation (Acheson, Stein & Swartzwelder, 1998; Zeigler et al., 2005). This underscores the importance of programs focused on alcohol and polysubstance use prevention for early college students, but to create effective alcohol and polysubstance use prevention strategies, it is also important to determine risk factors for use. Increased alcohol use and misuse during college years is often due to novel environmental influences, such as an increased workload, newly experienced independence, and easy access to alcohol (Martin et al., 2021). Other environmental and genetic risk factors for substance use include students' exposure prior to entering college. This may include family use, genetic vulnerability, and preexisting personality traits (Martin et al., 2021).

Alcohol use is also heavily intertwined with polysubstance use amongst college students. Polysubstance use, defined as using alcohol with any other mind-altering or physically altering substance, is a growing concern among traditional-aged college students. The prevalence of polysubstance use has risen at US colleges in recent years (Miech et al., 2026), and locally, more than a quarter (25.68%) of surveyed students from the University of North Carolina at Chapel Hill (UNC-Chapel Hill) reported polysubstance use in 2025 (CCC, 2025). Determinants of polysubstance use include adverse childhood experiences and depressive symptoms (Bajracharya et al., 2026). Further, polysubstance use elevates risk for negative outcomes, including unemployment after graduating (Welsh, Shentu & Sarvey, 2019). Further research is needed to identify factors that influence polysubstance use and its consequences among college campuses broadly, and specifically in the Chapel Hill community. The primary aim of this qualitative study is to understand how UNC-Chapel Hill students engage with polysubstance use and its impact on students and the broader Chapel Hill community.

Methods

This study used a rapid qualitative design involving semi-structured focus groups with a diverse sample of undergraduate and graduate students at UNC-Chapel Hill to explore perspectives and experiences with alcohol and other substances. This study was determined to be exempt from the Institutional Review Board review because it did not constitute human subjects research.

Participants

Participants were recruited via announcements in undergraduate and graduate classes, through posted flyers around campus, and social media (Instagram, GroupMe, etc) via existing university social channels. Those interested completed a brief survey on Qualtrics to ensure they

met the eligibility requirements. Eligibility criteria included (1) enrollment at UNC-Chapel Hill as either a graduate or undergraduate student (full-time or part-time) and (2) being 18-29 years old. The survey also captured basic demographic and personal information, including major/program, year in school, and contact information. It should be noted that while many sources standardize the college age range as 18-25, our study extended that range to 29 years old to capture more graduate students and diversify perspectives.

Procedures

Four focus group sessions were conducted from April 1 to April 13, 2026, and used a semi-structured interview guide. Each focus group session had two or three facilitators present, with one serving as the primary facilitator and the other(s) as secondary facilitators and note-takers. Each session was recorded for transcription and lasted between 51 and 65 minutes, with an average session length of 58 minutes. A total of 18 individuals participated in one of four focus group discussions, including 7 undergraduate and 11 graduate students. All focus group sessions included both undergraduate and graduate students. There were 4 focus groups with the following numbers of participants: 4**,** 7**,** 3**,** and 4. Sessions were held at central locations on the UNC-Chapel Hill campus using a hybrid model, with one participant joining via Zoom for three sessions. First, participants provided consent to participate and were briefed on the study's intended purpose. Participants who attended the focus group sessions received a $20 virtual gift card incentive through Tango, an electronic gift card distribution service.

Analytical Methods

Audio recordings from the focus group were transcribed using Otter.ai to create accurate records of participant responses. Transcripts were then reviewed and cleaned to remove identifying information, ensure confidentiality, and fix inaccuracies. The study used a Rapid Qualitative Analysis (Rapid Qual) approach to efficiently identify key themes and insights. This approach used a structured note-taking template with predefined domains aligned with the interview questions. A matrix of key quotations, organized by domain and focus group sessions, was developed, along with corresponding key themes and a summary of each focus group. The team then compared thematic summaries across all four focus groups to identify overarching findings and differences between groups. This approach enabled the timely synthesis of qualitative data while maintaining a systematic and rigorous analytical process.

Results

All four focus groups provided meaningful insights into the alcohol and polysubstance use landscape among UNC-Chapel Hill students, including key findings on the substances participants reported encountering or using (Figure 1) and those most frequently used in combination with alcohol (Figure 2).

Figure 1.

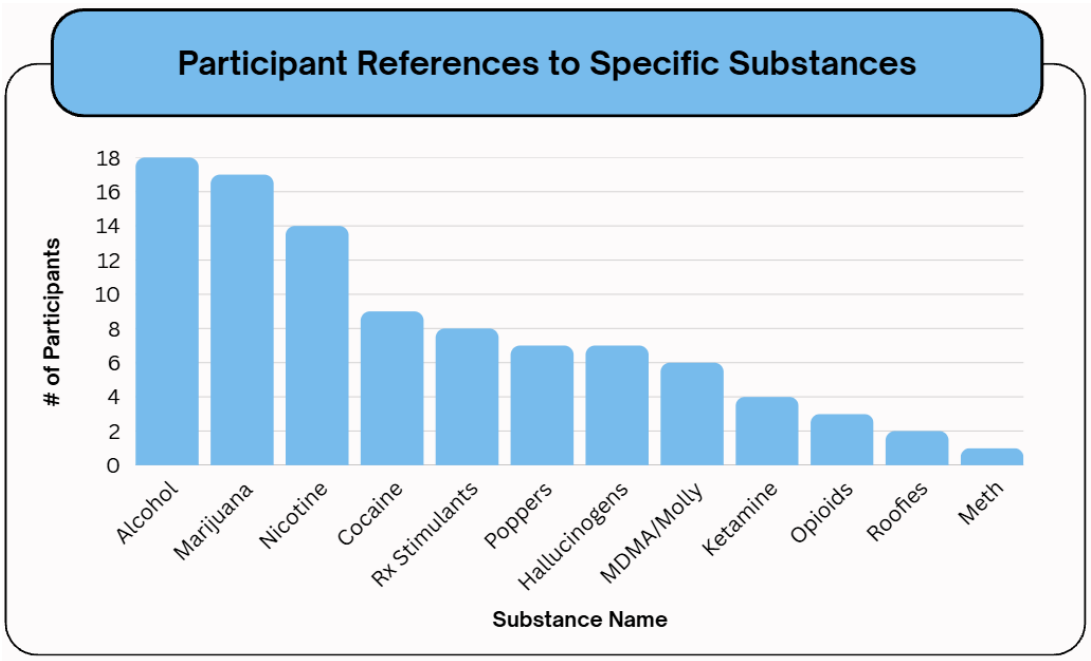


Figure 1. Number of participants who talked about engaging with, witnessing, or hearing about UNC students engaging in alcohol or single-substance use, stratified by the substance that the participant mentioned.

Figure 2.

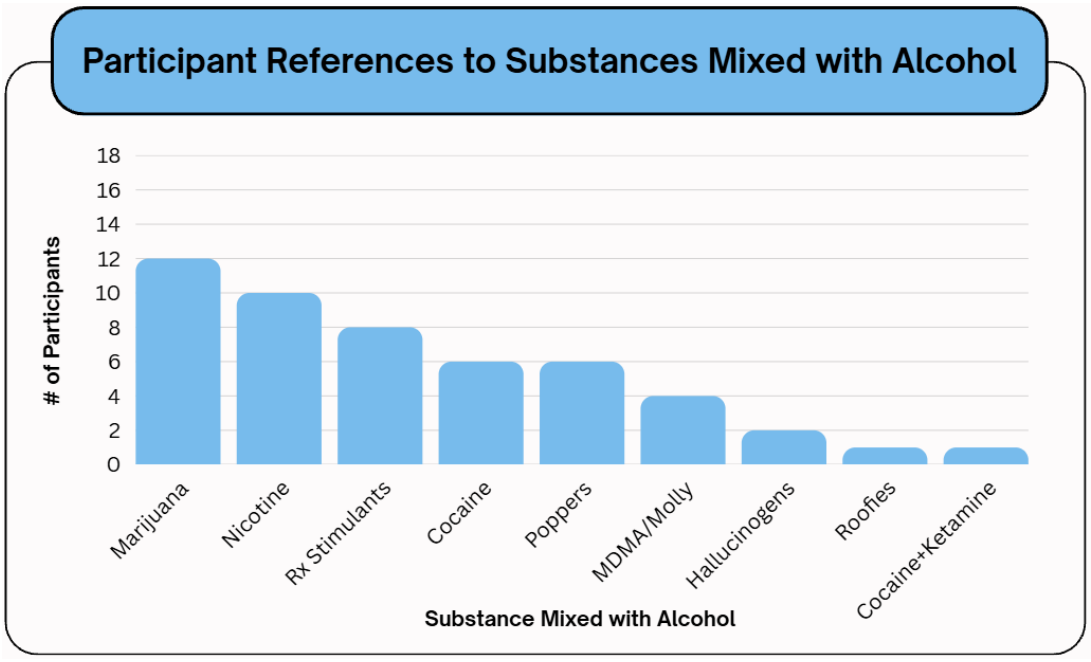


Figure 2. Number of participants who talked about engaging with, witnessing, or hearing about UNC students engaging in polysubstance use, stratified by the substance that was mixed with alcohol (n=18).

Following the Rapid Qual procedures as mentioned above, six overarching themes emerged from the focus group data, with some themes containing distinct subcategories that will be explored in this section.

Subjects Commonly Discussed

"There's much less of a safety culture around [drugs], and I do think that that is because there has been a more recent push in alcohol education, and especially towards the women, to make sure that you're always watching your drink. If you put your drink down, you should pick it back up if you know that someone has not been watching it safely the whole time. Whereas, because drugs are still super taboo, there is no real education about that, and people are failing to take into account that in college, people are probably going to do drugs at some point."

The co-use of alcohol and at least one other substance was commonly described across the focus groups. Rather than being something that was always planned ahead of time, participants often described it as something that became more likely once drinking had already begun. One participant explained that after someone is already intoxicated, it becomes "easier once you've got that initial intoxication [to say], 'why not?'" Participants also described how alcohol often acts as a starting point that lowers inhibitions and makes additional substance use feel easier to accept in the moment.

Across groups, marijuana and nicotine were the substances most consistently associated with alcohol use, with cocaine also frequently mentioned and ranked among the top three. Participants described marijuana as visible and common in Chapel Hill, with one participant noting they could smell their roommate smoking it. Furthermore, other participants said they had seen a lot of marijuana use among peers, especially in downtown Chapel Hill on Franklin Street. Nicotine was also described as normalized in drinking contexts, with one participant explaining, "When I'm drinking, it doesn't count," referring to using nicotine more casually while under the influence of alcohol. Participants also discussed prescription stimulants, cocaine, MDMA, hallucinogens, ketamine, and poppers, although these varied depending on social circles and environments. For example, one participant shared that they had friends who distributed Adderall, while another participant noted that cocaine was more common than they expected after arriving in Chapel Hill. Beyond this, participants pointed to differences in where certain substances show up. Poppers were described as more common in specific social spaces, specifically within queer communities, suggesting that patterns of use are also shaped by social and cultural context.

A major finding was the difference between alcohol education and broader substance use education. Participants recognized that alcohol safety messaging exists, but many felt there was much less open and destigmatized conversation around substance and polysubstance use. Several participants described polysubstance use as something that is already occurring in everyday

campus life, despite not having the knowledge or resources to understand or manage the risks involved fully.

Demographic Patterns

"Just because of the cultural differences that you start drinking [in Latine cultures] a lot earlier, especially because of the drinking age…Drinking age is 18, but like, your parents started giving you alcohol a lot before. I'm from Colombia, so I have lived that personally, like they were all offering [alcohol] since I was 15."

"I know in queer spaces, poppers are very popular."

As focus groups progressed, participants often discussed how different demographics or group characteristics experience and relate differently to alcohol and substance use. For example, one Latine participant described how alcohol is often introduced at an earlier age because the drinking age is lower in many South American countries, noting that some Latine cultures are less strict than American cultures and that some parents may offer alcohol to adolescents as young as 15 years old. This contrasts with their college experience, where they were surprised to encounter American peers who had never consumed alcohol before. Adding to this, participants consistently highlighted how early exposure, family and friends, and cultural norms can shape expectations and behaviors around drinking.

Furthermore, several participants referenced a perceived association between queer social spaces and the use of poppers, suggesting that certain substances may be more normalized within specific community contexts. In contrast, many participants who did not state engaging with queer spaces had never heard of poppers at all. The discussion around poppers was not explored in depth. Continuing the results around specific groups being associated with specific substances, fraternity brothers and business majors were associated with using cocaine, and students who attended raves were associated with using MDMA.

Rite of Passage

"It seemed like a kind of rite of passage and indication of, like, adulthood. And so I think that's the kind of pressure that I experienced with, like, oh, like, this is the signifier that I'm like, you know, I'm grown or whatever. "

A prominent theme across focus groups was alcohol and substance use acting as a rite of passage into adulthood. Participants discussed how drinking and using substances is a signifier of adulthood and being grown up. This framing prompted further discussion of first-year students as a potentially higher-risk group for substance use, as participants noted this group often feels pressure to engage in excessive or unsafe consumption to "keep up" with peers. Relatedly,

participants also alluded to underclassmen being more likely to consume beyond their personal limits with both alcohol and other substances due to increased pressure to get the full "college experience" and build their social networks early. Participants mentioned that a perceived level of "social status" comes with underclassmen maximizing intoxication, and especially first-years.

This theme is not the same for undergraduate and graduate students. Undergraduate participants often characterized alcohol use in a competitive or performative way, reinforcing its association with maturity and social status. In contrast, graduate participants reflected that although they may have previously shared these views during their undergraduate years, they no longer perceive alcohol or substance use as indicators of adulthood nor as the central component of their social interactions. Graduate participants reported they did not frequently take a second look at anyone who wasn't participating in substance use during social events, unlike during their undergraduate years, when they reported it felt as if everyone took note of their consumption levels. Instead, graduate participants explained that it was socially expected that substance use would decrease upon entering a graduate program.

Substance Use Culture Among Fraternities

"But I feel like the later into the night it gets, the more likely you're going to see Molly, the more you're going to see poppers. And I feel like that's also, again, just being in a social setting, where I feel a lot of these guys, you can tell they're trying to one-up each other, and each one wants to be the coolest guy there."

Fraternities were a recurring topic across focus group discussions. Participants consistently described a distinct social environment surrounding polysubstance use within these spaces. For example, one participant directly stated they associated cocaine with fraternities, and others emphasized how intense this environment can be in terms of alcohol and polysubstance use. Not only was cocaine directly associated with fraternities, but it was also perceived to be associated with the predominantly white fraternity contexts, with one participant noting that white men in fraternities taking cocaine is seen as a symbol of status, including popularity and wealth. Within the party environment, participants reported that fraternity events often begin with alcohol consumption and then lead to substance use later on in the event, including cocaine, MDMA, and ketamine. Participants also highlighted how substance use is often influenced by social dynamics such as competition and the perceived pressure to match or exceed peers' behaviors. For example, one participant recalled an instance in which they had seen someone choose to take MDMA simply because their friend didn't want to appear weak or be less involved than everyone else. This dynamic was further linked to perceptions of popularity and prestige within fraternity social circles, with one participant noting that fraternity brothers want to be the "coolest guys" in the room and the "first guy" to do something.

It is important to note that little information was provided regarding polysubstance use within the sorority social space, with one participant stating they would not elaborate further.

Decisions to Use

"I feel like a lot of my peers, I see make decisions about which of these they're going to engage with based on what other people are doing, or based on what percentage of other people are also doing it"

"The thing—the situations where I've seen more pressure is with crossing and with using weed with alcohol."

Across focus groups, many factors played a role in participants' decision-making process to drink alcohol and engage in polysubstance use. Specifically, participants described decisions to use alcohol and other substances as deeply tied to social context. Rather than framing use as a purely individual choice, participants often described their decisions as influenced by who they were with, where they were, what was available, and what others around them were doing. One participant explained, "There's kind of an unspoken expectation that there will be drinks there, there may or may not be substances there, and if everyone else is engaging in it, then why shouldn't you?" This quote reflects a broader pattern across the focus groups: participants were not always directly pressured, but social norms still shaped their choices through self-imposed pressure.

Pressure was instead often described as non-verbal, subtle, and self-imposed. In fact, participants pushed back against the idea that peer pressure usually looked like someone forcing a drink into your hand. Instead, they described pressure as feeling uncomfortable being the only one not drinking, or feeling like they were missing out. One participant explained, "It's not like peer pressure, it's more FOMO." Another participant explained that being around others who were drinking made them feel uncomfortable if they were not also participating.

Furthermore, group size and behavior also shaped the participants' discussions around decision-making. In small groups, participants reported feeling more comfortable not engaging in alcohol or substance use due to stronger trust and familiarity with peers. This comfortability also appeared in large groups; participants described feeling less singled out due to fewer people paying attention to their consumption behavior. In contrast, mid-sized groups were described as experiencing the highest pressure because the group was neither large enough to blend in nor small enough to feel comfortable explaining their consumption behavior. One participant explained that in a mid-size group, people may ask, "Why don't you want to be the only person not doing it?" and they may have to answer this numerous times throughout the night due to the visibility.

Group behavior also plays a role in the decision-making process. One participant described how group shots create pressure because "everybody's taking a shot," making refusal feel socially awkward, especially when the substance is already bought or poured for you. Graduate participants specifically noted that they typically decide not to drink at the beginning of

the social event. Still, when group shots are offered, they end up accepting to be respectful and considerate of others' money or efforts to create a "fun" environment for everyone.

Beyond this, participants described a more interpersonal form of pressure in which decisions were influenced by relationships rather than by group dynamics alone. One participant shared that her friend accepted an edible from another friend: "[She] said, 'I didn't even want to. I just didn't want to hurt her feelings.' That's where I've seen the most pressure. No one wants to be the only one not participating." This shows that pressure to use isn't necessarily obvious or direct, but often comes from not wanting to make others uncomfortable in the moment, especially when it is a close friend.

Beyond pressure, the accessibility of substances also plays a significant role in participants' decision-making. Undergraduate participants, in particular, frequently noted that they paid little attention to where a substance came from or its quality, and instead took anything they could find. One participant mentioned they found a random bag of weed in front of their apartment, and they smoked it due to their need to get out of a depressive state, despite knowing it was a poor decision. Another participant mentioned how they are more willing to participate in polysubstance use when there are other substances readily available, stating, " You just kind of like, 'Oh, I've got some shrooms.' Like, we could throw that in." Participants frequently noted how the accessibility of weed and THC products on Franklin Street makes it difficult to turn down and encourages them to participate in polysubstance use, especially after already consuming alcohol.

Furthermore, participants described how the disinhibition from drinking alcohol changes the decision-making process once drinking has already started. They emphasized how drinking made it easier to keep drinking or to use another substance. One participant stated, "When I've had more drinks, it's easier to convince me to take another one." Another explained that people may be more likely to use substances while drinking because "You can't really stand your ground as well as if you were sober". Other participants noted they don't often go into the night planning to consume multiple substances, but it happens based on how much they had to drink and what the people around them are doing.

Overall, decisions to use were shaped by a combination of social norms, access, peer behavior, and level of intoxication. Participants were not simply making isolated choices but were navigating real-time social environments in which saying "yes" often felt easier than saying "no."

<u>Prevention and Education Instead of Punishment</u>

"…understanding that while it is illegal–while it has had a lot of negative health effects—understand that use and abuse is going to happen on campus, off campus, and that it needs to be addressed, rather than just strictly forbidden and turning a blind eye to it."

Across all focus groups, the idea of education and prevention rather than punishment emerged consistently. Participants emphasized that programs focused on education, prevention, and safer use are important for creating a safer environment around substance use at UNC-Chapel Hill.

Several groups mentioned the Good Samaritan law, and participants noted that it creates a sense of safety when helping peers in emergency situations without fear of disciplinary consequences. One participant shared that this type of protection had directly impacted them. The Carpe Diem program, a 15-hour alcohol education course offered by UNC-Chapel Hill, was also discussed as a more constructive alternative to punishment, as it focuses on reducing risk rather than penalizing behavior.

Party registration was another program discussed. Participants noted that perceptions of party registration have shifted over time, changing from skeptical to positive. One undergraduate described it as "generally positive," explaining that it helps keep parties safer and reduces the risk of legal trouble. This participant explained that registering for a party doesn't make anyone less likely to attend and has therefore proven to be an effective prevention effort without disrupting participants' typical social patterns. In contrast, graduate participants reflected that when they were undergraduates at UNC-Chapel Hill, party registration was viewed more negatively, with concerns that it might increase the likelihood of law enforcement involvement.

In contrast to these positive views toward diversion and prevention approaches, participants raised concerns about punitive responses. One participant described an incident where individuals were cited for being in a room where alcohol was present, even if they were not participating in drinking. They said, "even people who weren't drinking in the room got cited… guilty by association." Experiences like this suggest that punitive approaches may discourage honesty and reduce the likelihood that participants will seek help in risky situations.

Throughout the discussions, many participants, particularly young women, described using informal strategies to stay safe, including relying on a "buddy system," moving in groups, using rideshare services, and not leaving drinks unattended. These practices reflect an existing awareness of risk and a desire to reduce harm, even outside of formal programs.

A consistent theme among undergraduate and graduate participants was the importance of harm reduction and stigma reduction. Participants did not describe substance use as something that could realistically be eliminated from campus life. Instead, they emphasized that it is already happening and that addressing it openly is more effective than ignoring or strictly punishing it. Many participants had heard of harm reduction tools such as Naloxone, but undergraduate participants in particular had less familiarity with how to access or use these resources.

The clearest perceived gap in substance use harm reduction was around education for substances beyond alcohol. Participants felt that alcohol education was more visible, while substance use remained more taboo and less openly discussed.

Overall, these findings point toward building and maintaining an institutional approach grounded in trust, harm reduction, open communication, and normalization of substance use beyond only alcohol. Participants suggested that students are more inclined to engage with

resources when they are framed around safety and support rather than discipline and are designed to be easy to apply.

Discussion

Alcohol and substance use among adolescents and young adults continues to be a significant public concern, as early initiation is linked to a higher risk of developing substance use disorders and experiencing a range of adverse outcomes (Florimbio et al., 2023). When analyzing the most common themes that emerged across focus group sessions, the results were discussed with an approach that prioritized creating effective, tangible, and specific recommendations for alcohol and polysubstance use prevention and harm reduction within the Chapel Hill community. Together, these findings informed the development of targeted recommendations to reduce harm while acknowledging the realities of students' social environments.

Analysis also revealed differences and similarities in how participants perceived and experienced alcohol use on its own compared to polysubstance use. The discussion is structured to highlight these differences. Specifically, social patterns and stereotypes appeared differently when referring to alcohol use on its own. This reflects the broader normalization of alcohol use within college settings, where it is often viewed as socially acceptable and expected.

Alcohol use was also identified as a factor promoting polysubstance use due to disinhibition, highlighting a key difference. In contrast, themes like the social construction around decision-making and harm reduction efforts appeared similarly when discussing both alcohol use alone and polysubstance use. Importantly, these perspectives were consistent among both undergraduate and graduate students, indicating that such patterns are influenced by wider campus norms rather than a specific student group.

Alcohol as a Driver of Polysubstance Use Through Disinhibition

Throughout all four focus groups, polysubstance use emerged as a dynamic and context-dependent behavior rather than a strictly planned or intentional practice. Participants often described substance use as developing throughout a social event, with their initial consumption, namely alcohol, influencing subsequent choices. A key mechanism underlying these findings is disinhibition. Participants consistently described how once alcohol consumption began, their decision-making shifted in ways that made polysubstance use feel more acceptable and less risky. Rather than entering social settings with the intention to engage in polysubstance use, many described how being “already tipsy” or “already drunk” lowered their guard and made it easier to justify use of another substance. In this way, alcohol does not just coexist with other substances but rather alters the conditions under which decisions are made, increasing susceptibility to both social influence and risk-taking behaviors, a theory commonly referred to as alcohol myopia (Karlsson et al., 2022). This highlights that polysubstance use is not simply about access or intention, but about how alcohol changes perception, judgement, and social responsiveness in real time.

Based on these findings, effective polysubstance use interventions should move beyond a sole focus on individual decision-making and preparation before beginning consumption, and instead address the role alcohol plays in disinhibition and prepare students for how they will make decisions once consumption has already begun.

Social Patterns of Polysubstance Use

Based on our findings, it is clear that polysubstance patterns differ between social contexts. A particular context that shapes patterns of use is around fraternity. The substances mentioned in the context of fraternities, besides alcohol, included MDMA, cocaine, and ketamine. Consistently, people described fraternities as hot spots for higher-risk use behaviors because many parties were fueled by social competition. Many people described witnessing people consistently 'one-upping' each other and using substances to be 'cool,' which suggests that fraternity members and potentially other party attendees view each other as competition and use the consumption of alcohol and other substances as a metric for social status. Therefore, the more willing a member is to participate in polysubstance use, the more popular they may appear.

Additionally, while participants did not go in depth with this discussion, it is worth noting the association of queer spaces with poppers. Even on a surface level, this suggests that there is a specific polysubstance use culture in the queer community. This inference is backed up by data that suggests more queer populations engage in substance use and misuse than non-queer populations. For example, queer folks are twice as likely to use illicit substances and almost twice as likely to experience a substance use disorder (AAC, 2024).

Given how social context shapes polysubstance use, interventions should be tailored to the specific needs of each group. Additionally, more detailed data on specific polysubstance use cultures should be collected to better inform interventions.

The Social Construction of Decision-Making

Focus group discussions revealed that decision-making around both alcohol and polysubstance use is deeply rooted in social constructs, norms, and expectations. Participants repeatedly emphasized that peer influence was not often verbal or direct, but rather occurred in an internalized perspective that presented itself more as a 'fear of missing out.' This fear was felt more commonly in mid-size groups rather than small or large groups, where they felt they were less visible and there was less expectation to partake in substance use. This suggests that group size plays an influential role in students' decision to partake in alcohol use. Participants reported feeling more visible in mid-size groups, and therefore felt more pressure not to be the only one not participating, suggesting that prevention programming should prioritize how students can approach decision-making, specifically in group settings where they feel more visible.

Many participants also described instances where they used substances, not because they personally felt uncomfortable abstaining, but because they did not want their peers to feel alone in participating. This finding displays participants' willingness to use substances in an effort to support others around them and help them feel more comfortable in their decisions. Some

participants felt that if another person had used a substance without any negative consequence, they felt safer to also use the substance, suggesting that peer consumption is used as an ineffective harm reduction strategy. Therefore, prevention efforts must consider not only how to respond when feeling individual discomfort from abstaining, but also the social dynamics that may encourage substance use. While prevention programs typically educate individuals on how to resist pressure when they feel uncomfortable being the only one not using substances (Griffin and Botvin 2010), these findings suggest the need to also prepare students with strategies for responding when they personally feel compelled to participate for the sake of others. This may include reframing what it means to "make someone feel not alone," as students can offer alternative ways to support friends participating in substance use without giving in to it themselves.

Harm Reduction and Preference Toward Non-Punitive Approaches

Throughout the focus group discussions, participants were asked about safety measures that UNC-Chapel Hill students take to reduce the harms associated with higher-risk alcohol and polysubstance use behaviors. Participants discussed the 'buddy system,' arriving and departing together, and ensuring peers are aware of their drinks at all times. While many of these strategies rely on trust among peers, they are not always adopted as regular habits, especially in contexts involving higher-risk alcohol and polysubstance use. These approaches can also fail if peer trust does not prompt friends to consistently safeguard one another when a higher-risk event occurs. While no participants directly mentioned purposefully leaving their friends in high-risk situations, one participant discussed a burdensome friend who consistently engaged in binge drinking and polysubstance use. The participant stated, "He was lucky that he had such a good group of people like us, because he could have died so many times," and directly referenced peer trust as the only safety net in these high-risk situations when he stated, "If one of those times we… weren't as good [of] people, then who knows." This conversation weakens the fidelity of the 'buddy system' and should be addressed at the institutional level to add more safety-net solutions for higher-risk behavior.

Other harm reduction approaches highly favored by participants include non-punitive policies and diversion programs, such as the Good Samaritan Law and Carpe Diem. Carpe Diem is a 15-hour diversion program developed in partnership with UNC Chapel Hill, that is specifically designed for university students who have been charged by law enforcement with a drug or alcohol violation and are eligible for first-offender deferred prosecution. The program takes a non-judgmental, educational approach, focusing on risk reduction, substance use awareness, stress management, self-awareness, and healthier coping strategies (*Carpe Diem Home*, n.d.). These were consistently raised throughout the focus group discussions, with participants noting that they promote trust in the institution.

In contrast, punitive approaches seemed to erode trust. This was evident in the tone of one participant who talked about a group of friends who were cited for drinking alcohol in the dorm rooms. The participant was upset that even the students who did not drink alcohol during

that event were "cited," because that implies that the students have two options: either they can tell authorities/supervision that their friends were drinking, or they will suffer the consequences. Approaches like these erode trust in the institution and are in direct contrast to policies and programs that promote non-punitive, positive behavior change. This suggests that interventions should be focused on diversion and harm reduction where appropriate to build trust within the UN-Chapel Hill community, and ultimately increase safety around alcohol and polysubstance use.

Undergraduate vs Graduate Perceptions

Clear differences emerged between undergraduate and graduate participants' perceptions and use of substances. For graduate participants, alcohol was typically integrated into social or professional settings in the background rather than as a defining component, resulting in less consumption. Whereas undergraduate participants described alcohol as the main focus of social events, resulting in higher consumption. For example, an undergraduate participant noted, " I don't think we see people taking casual drinks either," adding on, "If you go, you're going hard—seven shots." This difference in alcohol use translated into differences in how students utilize their weekends. Graduate participants framed weekends as a time for rest and preparation, whereas undergraduates more commonly described them as opportunities for stress relief through bingeing behaviors. Decision-making patterns also differed between groups. Graduate participants emphasized more deliberate, harm reduction-oriented strategies such as considering quality, source, and safety of the substance. A graduate participant noted that they believe regulated markets are safest and that they "order from brands that have COAs or certificates of analysis." These participants also noted choosing substances that were most discreet, especially in social settings with more professional individuals. Undergraduate participants, however, described a more experimental decision-making process in which they often chose the substance most immediately available to them, with little prior consideration of its contents or source.

The main similarity that emerged across participant groups was the presence of internalized, non-verbal pressure that shaped decision-making. Participants explained that they rarely experience someone verbally pressuring them to consume, but rather felt 'FOMO' (fear of missing out) if they didn't. Yet, undergraduate and graduate participants experience this non-verbal pressure differently, with graduate participants explaining it came from a desire to feel comfortable in a social situation, and undergraduate participants expressing that it came from more of a competitive perspective, as they felt like they should keep up at the same pace as friends, for example. Additionally, both undergraduate and graduate participants unanimously agreed that prevention-based programming should be used over punishment-based strategies to reduce alcohol and polysubstance use and misuse. Both groups were very interested in the health impacts that alcohol and other substances may have on the body, suggesting a health-based prevention program could be very effective.

Overall, these findings suggest that graduate participants tend to prioritize more intentional harm reduction strategies and demonstrate greater contextual awareness than

undergraduate participants. This may be driven by increased role complexity and more future-oriented thinking, in which professional identity and long-term consequences carry greater weight. In contrast, undergraduate participants see alcohol as central, participate in more experimental and immediate decision-making processes, and prioritize social expectations. These findings highlight a social environment in which experimentation, peer dynamics, and appearance to a social circle play a larger role in shaping behaviors around substance use.

Distinctions and similarities between undergraduate and graduate students' decision-making processes have important implications for targeted intervention strategies. Specifically, prevention and harm-reduction strategies should be tailored to the student level based on social patterns that commonly influence substance use decisions. While graduate participants exhibited greater harm-reduction-based decision-making, it is still necessary to invest effort in improving their consistent application of these strategies across all scenarios. Graduate participants' emphasis on selecting the most discrete substances, based on what is socially appropriate or most professional for the given environment, may at times conflict with harm reduction goals. This suggests that harm reduction strategies are being filtered through what is considered to be socially acceptable, which ultimately decreases their effectiveness.

Recommendations for the Campus and Community Coalition

The thematic findings and synthesis across focus groups led to the development of actionable recommendations that the Campus and Community Coalition can use to implement policies or programs that aim to prevent and reduce the harm from polysubstance use. One way that the recommendations were sourced was through asking the focus group participants how they would address polysubstance use among UNC students, of which several participants provided feasible, actionable approaches. The participant-offered approaches include (1) updating the first-year and transfer student orientation alcohol modules to include other substances and polysubstance use, (2) maintaining and expanding non-criminalizing diversion programs such as Carpe Diem, (3) making counselors who specialize in alcohol and substance misuse more accessible to students, and (4) normalizing harm reduction tools and strategies around and among UNC campus and students.

After reviewing these recommendations, analyzing the current gaps in polysubstance use safety among UNC students, and reviewing the literature on these, there are several evidence-based approaches to consider. Lack of awareness and stigma toward harm reduction tools and approaches were a common theme throughout the focus groups, and supported in the literature (Yen et al., 2025). Yen et al. found that among students at a college in the southern US, more than half of survey respondents were unlikely to carry Naloxone themselves, but more respondents were likely to recommend a friend do so. Furthermore, more than a third of respondents mentioned a lack of knowledge around harm reduction resources, and 78% of respondents were unaware of free Naloxone via on-campus pharmacies. This fundamental lack of awareness highlights the need to make harm reduction the default by increasing awareness and access to tools that reduce polysubstance use harm. Campaigns aimed at shifting the norms

around substance use and polysubstance use behavior have been shown to prevent or reduce the harm of higher-risk alcohol and substance-related behavior (Hembroff et al., 2021; Pischke et al., 2021), and the Campus and Community Coalition should consider adapting and implementing programs such as these. They should also consider other approaches to normalize harm reduction approaches on the UNC campus, including (1) increasing Naloxone and drug test strip access in high-visibility student areas, (2) normalizing carrying harm reduction tools as routine behavior, and (3) embedding this mindset into standard campus practice and expectations.

Finally, the Campus and Community Coalition should take steps to prevent or reduce the harm caused by events where the disinhibition of alcohol escalates to polysubstance use. This can be done by shifting norms around polysubstance use at the institutional level, starting with revamping the first-year and transfer student orientations about alcohol policies and behaviors on the UNC campus to include information about substance and polysubstance use. The modules should spread general awareness, and students should gain practical skills to better understand their decision-making, specifically how to respond when the opportunity to engage in polysubstance use is presented while under the influence of alcohol.

Limitations and Future Direction for Research

Limitations in this study include a limited time frame for data collection. This led to fewer focus groups being conducted. If more groups had been conducted, more diverse populations could have been included in the focus group discussion, bringing more diverse experiences and themes that are not currently represented in our sample. Additionally, due to participant attrition, the number of participants in 3 of the 4 focus groups was lower than the original goal of at least 8 per group. Because of this, we may have collected limited data on certain important aspects of substance use culture on UNC-Chapel Hill's campus.

Beyond structural limitations, in one of the focus groups, a participant struck the comparison of fraternities and sororities attitudes around alcohol and substance use. They noted how fraternities were more open about their alcohol and substance use in comparison to how secretive sororities could be. This participant believes the secrecy may come from Southern patriarchal standards about what constitutes a woman. Therefore, future research focus should be on how women in sororities navigate alcohol and substance use.

Additionally, queer demographics and substance use were mentioned on a surface level in multiple groups, but no in-depth discussion occurred. This could be due to a lack of queer participants, but the lack of discussion signals the need for more understanding as to how queer individuals navigate alcohol and substance use culture. This is particularly pressing in America's current sociopolitical climate, rife with human rights violations heavily felt by the queer community. As such, studies in the near future should invest in understanding how these sociopolitical circumstances affect alcohol and substance use culture in queer spaces to begin to help members of the community who may be impacted.

References

Acheson, S. K., Stein, R. M., & Swartzwelder, H. S. (1998). Impairment of semantic and Figural Memory by acute ethanol. *Alcoholism: Clinical & Experimental Research*, *22*(7), 333. https://doi.org/10.1097/00000374-199810000-00010

Bajracharya, S., Salwa, A., Hassan, A., Vujanovic, A. A., & Montemayor, B. N. (2026). Examining early adversity, polysubstance use, and depressive symptoms in college students. *International Journal of Mental Health and Addiction*. https://doi.org/10.1007/s11469-025-01634-8

Campus & Community Coalition Scorecard. Campus & Community Coalition (CCC). (2025). https://embed.clearimpact.com/Scorecard/Embed/82237

Carpe Diem Home. (n.d.). Retrieved April 29, 2026, from https://www.carpediemnc.org/Carpe-Diem-Registration.html

Florimbio, A. R., Coughlin, L. N., Bauermeister, J. A., Young, S. D., Zimmerman, M. A., Walton, M. A., & Bonar, E. E. (2023). Risky Drinking in Adolescents and Emerging Adults: Differences Among Individuals Using Alcohol Only versus Polysubstance Use. Substance Use & Misuse, 58(2), 211–220. https://doi.org/10.1080/10826084.2022.2152192

Griffin, K. W., & Botvin, G. J. (2010). Evidence-based interventions for preventing substance use disorders in adolescents. *Child and adolescent psychiatric clinics of North America*, *19*(3), 505–526. https://doi.org/10.1016/j.chc.2010.03.005

Hembroff, L. A., Martell, D., Allen, R., Poole, A., Clark, K., & Smith, S. W. (2021). The long-term effectiveness of a social norming campaign to reduce high-risk drinking: The Michigan State University experience, 2000-2014. *Journal of American college health : J of ACH*, *69*(3), 315–325. https://doi.org/10.1080/07448481.2019.1674856

Karlsson, H., Persson, E., Perini, I., Yngve, A., Heilig, M., & Tinghög, G. (2022). Acute effects of alcohol on social and personal decision making. Neuropsychopharmacology, 47(4), 824–831. https://doi.org/10.1038/s41386-021-01218-9 https://www.cdc.gov/stop-overdose/caring/polysubstance-use.html

Martin, K. P., Benca-Bachman, C. E., & Palmer, R. H. C. (2021). Risk for alcohol use/misuse among entering college students: The role of personality and stress. *Addictive Behaviors Reports*, *13*, 100330. https://doi.org/10.1016/j.abrep.2020.100330

Miech, R. A., Patrick, M. E., O'Malley, P. M., Jager, J. O., & Jang, J. B.(2026). Monitoring the Future national survey results on drug use, 1975–2025: Overview and key findings for secondary school students. Monitoring the Future Monograph Series. Ann Arbor, MI: Institute for Social Research, University of Michigan

Pischke, C. R., Helmer, S. M., Pohlabeln, H., Muellmann, S., Schneider, S., Reintjes, R.,

Schmidt-Pokrzywniak, A., Girbig, M., Krämer, A., Icks, A., Walter, U., & Zeeb, H. (2021). Effects of a Brief Web-Based "Social Norms"-Intervention on Alcohol, Tobacco and Cannabis Use Among German University Students: Results of a Cluster-Controlled Trial Conducted at Eight Universities. *Frontiers in public health*, *9*, 659875. https://doi.org/10.3389/fpubh.2021.659875

Welsh, J. W., Shentu, Y., & Sarvey, D. B. (2019). Substance Use Among College Students. *Focus (American Psychiatric Publishing)*, *17*(2), 117–127. https://doi.org/10.1176/appi.focus.20180037

White, A., Castle, I. J., Chen, C. M., Shirley, M., Roach, D., & Hingson, R. (2015). Converging Patterns of Alcohol Use and Related Outcomes Among Females and Males in the United States, 2002 to 2012. *Alcoholism, clinical and experimental research, 39*(9), 1712–1726. https://doi.org/10.1111/acer.12815

Yen, K. D., Shook, A., Marciniak, M. W., & Rhodes, L. A. (2025). College student attitudes and perceptions toward opioid overdose and harm reduction at a large, public university. JAPhA Practice Innovations, 100090. https://doi.org/10.1016/j.japhpi.2025.100090

Zeigler, D. W., Wang, C. C., Yoast, R. A., Dickinson, B. D., McCaffree, M. A., Robinowitz, C. B., & Sterling, M. L. (2005). The neurocognitive effects of alcohol on adolescents and college students. *Preventive Medicine*, *40*(1), 23–32. https://doi.org/10.1016/j.ypmed.2004.04.044

Van Horn, S. L., & Lipari, R. N. (2017, August 17). Children living with parents who have a substance use disorder. https://www.samhsa.gov/data/sites/default/files/report_3223/ShortReport-3223.html

Why substance abuse is higher within the LGBTQ community. American Addiction Centers (AAC). (2024, November 22). https://americanaddictioncenters.org/lgbtqiapk-addiction/why-substance-abuse-is-higher-within-the-lgbtq-community